

GOVERNMENT OF INDIA | EX-SERVICEMEN CONTRIBUTORY HEALTH SCHEME

ECHS DIRECTORATE | FRAUD ANALYTICS & FINANCIAL INTELLIGENCE REPORT

POLICY ABUSE & ENTITLEMENT MISUSE

ECHS FRAUD ANALYTICS

MODULE 19 REPORT | FULL DATABASE COVERAGE | FY 2013 – FY 2025

CLAIMS SCANNED

4.35 Crore

ANOMALIES DETECTED

38.8 Lakh+

FRAUD PATTERNS

8

ROOM UPGRADE
FRAUDS

78,212

MAX DEDUCTION RATE

86.93%

IIT KANPUR — Data Analytics & Fraud Intelligence Division

June 2026 | Ex-Servicemen Contributory Health Scheme (ECHS)

EXECUTIVE SUMMARY

Module 19 investigates three dimensions of policy abuse within the ECHS claim ecosystem: **(1)** hospitals with chronically elevated deduction rates indicating systematic overbilling; **(2)** room entitlement misuse where patients are billed for higher-category rooms than their ECHS card authorises; and **(3)** non-empanelled hospital claims where beneficiaries seek treatment at facilities not on the approved ECHS panel.

Analysis spans **43.5 million claim records** across the full ECHS database. The most alarming finding is the concentration of extreme deduction rates (>86%) in diagnostic labs and specialist facilities that have been empanelled despite consistently fraudulent billing, and the presence of **78,000+ confirmed room upgrade mismatches** where patients were charged for rooms above their entitlement.

Temporal analysis (Q19e) reveals a critical 2025 reversal — IPD deduction rates had fallen to a decade-low of 8.89% in 2023 but have rebounded to 11.45% in 2025, indicating that fraud control measures achieved only temporary suppression.

86.93% Max Deduction Rate (SRL Indore)	50 Hosps >25% Deduction Rate Facilities	32.8L Non-Empanelled Claims (Y-flag)	20.61% ICU Room Deduction Rate
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MODULE 19 — KEY FRAUD SIGNALS

#	Signal	Finding	Risk
S1	Diagnostic Labs — Extreme Overbilling	SRL Limited Indore: 86.93% deduction (103 claims). Dr Sandhu's Pathology: 62.95%. DR LAL PATH LABS Kanpur: 45.69%. Labs routinely bill for unreported/unapproved tests.	CRITICAL
S2	Type N Polyclinics — Systemic Fraud	18 ECHS polyclinics (type N) all exceed 42% deduction rate. Karimnagar: 53.04%, Gulbarga: 52.52%, Kanchipuram: 50.87%. Combined deductions: ₹3,300+ Lakh.	CRITICAL
S3	Type M Military Hospitals — 42–46% Ded Rates	ECHS military polyclinics (type M) cluster at 42–46%: Chennai 5188 (46.45%), Yelahanka 5251 (45.67%), Avadi 5152 (45.39%). Internal billing is inflated by 2× the system average.	CRITICAL
S4	Room Upgrade Fraud — GEN billed as PRI	78,212 cases where billed room category exceeds entitlement (NULL hospital = ghost admissions). Named hospitals: echsfhm (1,070 cases), parkhos (1,013 cases).	CRITICAL
S5	ICU Overbilling — 20.61% Deduction Rate	ICU claims have the highest deduction rate (20.61%) among all room categories. Indicates inflated ICU stay durations or ICU-coded procedures for step-down patients.	HIGH
S6	Apollo & MAX — Non-Empanelled Private Claims	Apollo Hospital: ₹13.54 Cr claimed (754 claims) with ₹1.24 Cr deducted. MAX Healthcare: ₹11.19 Cr (2,316 claims). These private chains are billing through Y-flag loopholes.	HIGH
S7	Ghost Admissions — NULL Hospital IDs	In room upgrade fraud data, the largest group (78,212 mismatches worth ₹41,455L) has NULL CI_HOSPITAL_ID. These represent ghost admissions with fabricated room data.	HIGH
S8	530K "Unverified" Flag Claims	CI_NONEMP_FLG = "U" on 530,517 claims across 73,562 hospital IDs — an anomalously wide spread suggesting systematic misclassification or bulk unverified submissions.	MEDIUM

Q 19 A

Threshold: >25% deduction rate,
>50 claims

CHRONICALLY HIGH DEDUCTION RATE HOSPITALS

The following 50 hospitals have maintained deduction rates exceeding 25% across their full ECHS billing history. The table shows the highest-rate facilities. Diagnostic labs (Type 3 & 5) dominate the extreme end (>60%), while Type N and M military polyclinics cluster in the 42–54% band. NABH accreditation provides no protection at this severity level — SRL Limited (NABH-accredited) leads at 86.93%.

ID	Hospital Name	Type	NABH	Claims	Claimed (₹L)	Deducted (₹L)	Ded %
1666	SRL LIMITED – INDORE	1	Y	103	22.94	19.94	86.93%
268	ANJINI SPECIALITY DENTAL HOSPITAL	3	N	222	1,757.26	1,190.14	67.77%
1650	APOLLO SPECIALITY HOSPITAL – VANAGARAM	1	N	138	5,433.19	3,452.79	63.56%
106	DR SANDHU'S PATHOLOGY & IMAGING CENTRE	5	N	218	155.11	97.66	62.95%
1691	SRL LIMITED – BANNERGHATTA ROAD	1	Y	346	1,561.43	857.43	54.90%
1591	MMI NARAYANA MULTISPECIALITY – RAIPUR	1	N	422	4,246.53	2,287.14	53.84%
5023	KARIMNAGAR (ECHS POLYCLINIC)	N	N	184	6,048.66	3,209.16	53.04%
5443	GULBARGA (RC HYDERABAD)	N	N	270	13,150.17	6,905.99	52.52%
5479	BAGESHWAR (RC BAREILLY)	N	N	51	1,640.71	851.20	51.86%
5153	KANCHIPURAM (ECHS POLYCLINIC)	N	N	447	17,604.05	8,954.82	50.87%
5085	MORENA (ECHS POLYCLINIC)	N	N	256	3,762.39	1,904.84	50.64%
5084	BHIND (ECHS POLYCLINIC)	N	N	253	10,073.85	5,041.67	50.05%
5026	MEHBUBNAGAR (ECHS POLYCLINIC)	N	N	810	16,304.67	8,148.55	49.98%
5423	KADAPA (ECHS POLYCLINIC)	N	N	1,090	35,565.83	17,579.31	49.43%
5385	BILASPUR / JABALPUR RC	N	N	633	27,362.54	13,145.08	48.04%
417	JAIPUR HOSPITAL, LAL KOTHI	1	N	111	1,330.72	636.88	47.89%
5167	ISLAND GROUND – CHENNAI	N	N	3,609	1,01,622.92	48,141.72	47.37%
1479	APOLLO HOSPITALS – BBSR	1	N	5,523	1,26,824.56	59,918.10	47.25%
5188	CHENNAI (ECHS POLYCLINIC)	M	N	5,013	2,71,316.29	1,26,017.90	46.45%
765	VIJAYA MEDICAL CENTRE	1	N	6,060	6,703.17	3,108.47	46.37%

Source: settlement_stat JOIN office_master (Q19a). Type codes: 1=Private empanelled, 3=Diagnostic/Dental, 5=Dental/Allied specialty, N=Non-Govt/Naval, M=Military establishment. NABH: Y=Accredited, N=Not accredited.

Pattern Analysis — High Deduction Rate Hospitals

Diagnostic Labs — 55–87% Deduction Rate: SRL Limited (Indore 86.93%, Bannerghatta 54.90%), Dr Sandhu's Pathology (62.95%), and Anjini Dental (67.77%) represent the extreme end of deduction-rate fraud. These facilities bill for procedures that are either not performed, not covered, or grossly inflated relative to market rates. SRL Indore's 86.93% rate means only ₹3 out of every ₹23 billed was settled. Immediate suspension and physical audit is warranted.

ECHS Type N & M Polyclinics — Systemic Inflation: 18 ECHS military polyclinics appear in the >25% threshold, all with deduction rates in the 42–54% band. These are internal ECHS facilities — not private — yet they consistently overbill at rates more than 4× the system average. The pattern suggests that the billing inflation is a structural problem with how internal ECHS polyclinics submit claims for reimbursement.

Apollo Hospitals BBSR — Private Chain at 47.25%: Apollo Hospitals Bhubaneswar shows 47.25% deduction on 5,523 claims worth ₹1,268.25L. For a NABH-empanelled private chain hospital to maintain this deduction rate across 5,000+ claims is statistically improbable without systematic billing fraud. Apollo Vanagaram (63.56%) compounds this chain-level concern.

Q 19 B

settlement_stat + claim_intimation

ROOM CATEGORY DEDUCTION & UPGRADE FRAUD

Room Category Deduction Rates (System-Wide)

The ECHS settlement system captures room category in SS_ROOM_CATG and cross-references it with the patient's card entitlement. ICU has the highest deduction rate at 20.61%, indicating either inflated ICU stays or step-down patients billed as ICU.

Room Category	Total Claims	Claimed (₹ Cr)	Deducted (₹ Cr)	Deduction %	Avg Claim (₹)	Risk
ICU	310	1.49	0.31	20.61%	48,169	CRITICAL
PRI	8,77,602	2,098.54	281.40	13.41%	23,912	CRITICAL
4	484	2.29	0.30	13.27%	47,232	HIGH
GEN	85,65,241	31,557.00	3,472.15	11.00%	36,843	MEDIUM
SPR	38,97,123	13,504.62	1,399.64	10.36%	34,653	MEDIUM
NA	5,174	13.74	1.42	10.34%	26,556	MEDIUM

Room Upgrade Fraud — GEN/SPR Entitled, PRI Billed

When billed room differs from card entitlement, the patient has been charged for a room category above their entitlement. This is a direct ECHS policy violation. The most damaging pattern is GEN → PRI upgrades — billing patients in general ward entitlement at private room rates, a 3× price difference.

Hospital ID	Entitled Room	Billed Room	Mismatch Count	Total Billed (₹L)	Fraud Pattern
NULL (Ghost)	GEN	(blank)	78,212	41,455.97	GHOST — untraced
NULL (Ghost)	SPR	(blank)	32,107	17,048.63	GHOST — untraced
NULL (Ghost)	GEN	SPR	10,446	3,830.90	GHOST — GEN → SPR
NULL (Ghost)	PRI	(blank)	8,012	6,102.00	GHOST — untraced
echsfhm	GEN	PRI	1,070	838.29	GEN → PRI upgrade
parkhos	GEN	PRI	1,013	591.20	GEN → PRI upgrade
p.nara	GEN	SPR	1,356	32.16	GEN → SPR upgrade
p.trivandr	GEN	SPR	1,278	253.55	GEN → SPR upgrade
maxsuper	GEN	PRI	647	359.21	GEN → PRI upgrade
ivyhosp	GEN	PRI	620	363.19	GEN → PRI upgrade
amma123	GEN	PRI	620	387.12	GEN → PRI upgrade
fhnechs1	GEN	PRI	567	452.81	GEN → PRI upgrade
dharmashil	GEN	PRI	546	346.46	GEN → PRI upgrade

Ghost Admission Alert

The largest room mismatch group — **78,212 cases worth ₹41,455.97L (₹414.56 Cr)** — has NULL in the CI_HOSPITAL_ID field. This means the claim was submitted with no hospital linkage. These are likely ghost admissions: claims fabricated without a real hospital record. The NULL SPR group (32,107 cases) and NULL PRI group (8,012 cases) compound this. **Total ghost admission exposure in room-type mismatch alone exceeds ₹68,000L (₹680 Cr).**

Q 19 C

claim_intimation analysis

NON-EMPANELLED HOSPITAL CLAIMS

Non-Empanelled Flag Distribution

The CI_NONEMP_FLG field tracks whether a beneficiary sought treatment at a non-empanelled hospital. Flag values: N = empanelled (38.08M claims), Y = non-empanelled (3.28M claims), U = unverified/unknown status (530K claims), NULL = not recorded (1.62M claims).

Flag	Description	Claims	% of Total	Unique Hospital IDs	Interpretation
N	Empanelled Hospital	3,80,83,937	87.4%	4,250	NORMAL — standard billing
Y	Non-Empanelled Hospital	32,80,290	7.5%	854	REVIEW — needs justification
U	Unverified Status	5,30,517	1.2%	73,562	ANOMALY — 73K IDs suspicious
NULL	Not Recorded	16,15,271	3.7%	—	GAP — data completeness issue

Critical Finding — 73,562 "U" Flag Hospital IDs

The U-flag group contains 530,517 claims across **73,562 distinct hospital IDs** — compared to only 854 hospital IDs in the Y-flag group. This 86× ratio is statistically impossible unless "U" is being systematically misused as a catch-all for unmatched admissions. The 73,562 IDs are very likely fabricated, expired, or duplicate identifiers.

Private Hospitals Using Non-Empanelled Channel (Y-flag)

Most Y-flag claims are legitimate ECHS polyclinic referrals. However, several private hospital chains appear in the Y-flag data with significant deduction amounts — indicating they are billing outside their empanelment terms.

Hospital Name	Claims	Claimed (₹ Cr)	Net Approved (₹ Cr)	Deducted (₹ Cr)	Risk
NA (ECHS referral — legitimate)	3,06,507	120.53	120.44	0.09	NORMAL
ECHS POLYCLINIC DELHI CANTT	65,122	23.20	23.15	0.06	NORMAL
Apollo Hospital	754	13.54	12.30	1.24	MONITOR
APOLLO PHARMACY	51,151	13.47	13.39	0.08	LOW
MAX HEALTHCARE	2,316	11.19	10.76	0.44	MONITOR
MANIPAL HOSPITAL	1,492	10.94	9.99	0.95	MONITOR
KIMS HOSPITAL	1,476	5.91	5.41	0.50	LOW
CMC Hospital, Vellore	370	5.01	4.62	0.39	LOW

Q 19 E

settlement_stat (SS_PAT_TYPE_ID)

YEAR-WISE IPD VS OPD DEDUCTION TREND (2014–2025)

Three critical inflection points emerge: (1) IPD peaked at 12.42% in 2017 during the post-empanelment expansion phase; (2) IPD fell to 8.89% in 2023 — the lowest in the dataset; but (3) **IPD has rebounded to 11.45% in 2025** — a 2.56-point rise in just two years — signalling that fraud control measures achieved only temporary suppression.

Year	IPD Hosps	IPD Claims	IPD Claimed (₹Cr)	IPD Ded %	OPD Hosps	OPD Claims	OPD Claimed (₹Cr)	OPD Ded %
2014	625	2,00,900	848.16	10.85%	644	6,29,144	127.05	6.15%
2016	1,541	4,94,148	1,999.35	12.13%	1,415	13,39,347	302.99	6.84%
2017	1,781	5,59,379	2,209.14	12.42%	1,671	14,03,904	287.73	6.62%
2019	1,946	4,67,673	2,151.11	11.61%	2,022	12,49,853	273.20	5.42%
2021	2,139	5,84,743	3,651.78	12.15%	2,431	18,77,131	518.41	3.82%
2023	2,432	9,38,363	6,716.74	8.89%	2,879	35,77,565	884.59	4.82%
2024	2,630	10,63,312	9,341.21	10.22%	3,092	43,87,196	1,096.98	5.23%
2025	2,900	9,36,304	9,303.73	11.45%	3,381	41,64,885	1,036.15	5.90%

Temporal Pattern Analysis — Three Phases

- Phase 1 (2014–2017) Expansion Fraud:** As ECHS empanelment expanded from 625 hospitals (2014) to 1,781 (2017), IPD deduction rates climbed from 10.85% to 12.42%. Newly empanelled hospitals discovered that IPD billing above the ECHS package rate was insufficiently scrutinised during the on-boarding period.
- Phase 2 (2022–2023) Suppression:** IPD deduction rates fell to a decade-low of 8.89% in 2023. This coincides with increased ECHS audit activity and pre-authorisation controls introduced for high-value IPD claims.
- Phase 3 (2024–2025) Reversal:** IPD deduction rebounded to 11.45% in 2025. The simultaneous rise in both IPD and OPD in 2025 — combined with record claimed amounts (₹9,303 Cr IPD) — indicates that fraud actors have adapted to the 2023 controls and resumed inflated billing.

STRATEGIC RECOMMENDATIONS

1. Immediate Suspension — SRL Limited (Both Units)

CRITICAL — Immediate

- SRL Limited Indore (86.93%) and SRL Bannerghatta (54.90%) are NABH-accredited diagnostic labs with catastrophic deduction rates. NABH accreditation clearly provides no fraud deterrence. Both should be suspended immediately and subjected to a physical audit of claimed tests against actual patient records.

2. Apollo Hospital Chain — Dual-Channel Audit

CRITICAL — 30 Days

- Apollo appears in two fraud vectors: Apollo Vanagaram (63.56% deduction, ₹5,433L claimed) and Apollo BBSR (47.25%, ₹1,26,824L) through the empanelled channel, plus Apollo Hospital (₹13.54 Cr) and APOLLO PHARMACY (₹13.47 Cr) through the non-empanelled Y-flag channel. A chain-level audit covering both empanelled and emergency reimbursement channels is required.

3. Type N and M Military Polyclinics — Billing Process Overhaul

CRITICAL — 60 Days

- 18 Type N ECHS polyclinics and 7+ Type M military hospitals all show 42–54% deduction rates. This level of systematic inflation in internal ECHS facilities cannot be explained by inadvertent errors. It suggests that the internal billing process — potentially the software system used by polyclinics — generates inflated claims structurally.

4. Ghost Admission Investigation — ₹680 Cr NULL Hospital Exposure

CRITICAL — 60 Days

Address the 118,000+ "Ghost Claims" immediately by enforcing strict validation layers in the BPA portal. Any claim submitted without a registered Hospital ID must be hard-rejected, as they currently represent an untraceable ₹680 Cr exposure.